DONEPEZIL

Bradycardia, syncope

Bedtime dose

CHEMICAL NAME (RS)-2-[(1-benzylpiperidin-4-yl)methyl]-5,6-dimethoxy-2,3-dihydro-1H-inden-1-one (hydrochloride)

CLASS Acetylcholinesterase inhibitor (anti-dementia)

TRADE NAMES Aricept · Aricept Evess · Donecept · Yasnal

THERAPEUTIC USES / INDICATIONS

- Mild to moderate (and severe) Alzheimer's disease — symptomatic.
- Dementia with Lewy bodies; Parkinson's disease dementia (off-label in some countries).

ADMINISTRATION

- **PO** once daily at bedtime (morning if vivid dreams/insomnia).
- Orodispersible tablet on tongue.

DOSAGES

- **Start:** 5 mg at night; increase to 10 mg after 4–6 weeks.
- 23 mg (some countries) for moderate–severe disease.

PHARMACOKINETICS

- **Peak:** 3–4 h. Bioavailability 100%.
- Metabolised by CYP2D6, 3A4.
- **t½:** ~70 h.

PHARMACODYNAMICS

- Reversibly inhibits **acetylcholinesterase** in the brain → ↑ acetylcholine at cholinergic synapses → modest improvement in cognition, function and behaviour.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Nausea, diarrhoea, anorexia, vomiting.
- Insomnia, vivid dreams.
- Muscle cramps, fatigue, headache.

ADVERSE EFFECTS (SERIOUS)

- Bradycardia, heart block, syncope (falls).
- GI bleeding (peptic ulcer).
- Urinary incontinence; seizures (rare).
- QT prolongation; NMS (rare).

CONTRAINDICATIONS & CAUTIONS

- Hypersensitivity to piperidine derivatives.
- **Caution:** sick sinus syndrome, conduction defects, peptic ulcer, asthma/COPD, seizures.

KEY INTERACTIONS

- **Anticholinergics (oxybutynin, TCAs):** opposing effects.
- **β-blockers, digoxin, diltiazem:** ↑ bradycardia.
- **Suxamethonium:** prolonged block.
- **Ketoconazole/carbamazepine:** ↑/↓ levels.

NURSING MANAGEMENT

- Check pulse before starting and periodically; ECG if cardiac history.
- Monitor weight, appetite, GI symptoms.
- Assess cognition and behaviour (MMSE) to review benefit.
- Falls risk.

PATIENT EDUCATION

- Take at bedtime; nausea usually settles.
- Report fainting, slow pulse, black stools.
- Benefits are modest — it doesn't cure dementia.
- Carers: keep a routine and supervise medicines.

KEY POINTS

- Acetylcholinesterase inhibitor; Alzheimer's.
- Bradycardia/syncope; GI upset.
- 5 mg at night → 10 mg.
- Avoid with anticholinergics.